Demo Healthcare Benefits Plan

A fictional plan for document ingestion, semantic search, and cited answers.

Plan identity

Plan ID: **DEMO-PLAN-001**
Document version: **1.0**
Effective period: **January 1, 2026 to December 31, 2026**
Currency: **USD**

Synthetic data notice

All coverage rules, amounts, limits, and administrative procedures in this document are invented for software testing. This is not an insurance policy, evidence of coverage, or medical advice. No real insurer or member is represented.

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Eligibility, network, and general rules

Eligibility and plan period

This demo plan applies only to an active synthetic member enrolled in DEMO-PLAN-001 on the service date. A service outside the effective period cannot be evaluated using this version. Coverage cannot be established from the document alone without checking enrollment and the service date.

In-network providers

The benefits stated in this document apply to in-network providers unless a section explicitly says otherwise. A provider must be verified in the applicable network directory for the service date. This PDF does not contain a provider directory or live network status.

Out-of-network services

Routine, non-emergency out-of-network services are excluded. Emergency care has separate handling: the member must contact the demo administrator after stabilization for review. This document does not define emergency cost sharing; do not infer it from routine visit rules.

Annual limits and remaining benefits

An annual limit means the maximum for one calendar year. Visit counts reset on January 1 and unused visits do not carry over. This document states plan limits, not a member's used or remaining visits. Remaining benefits require an eligibility or utilization record.

Terminology

Prior authorization means approval obtained before a service. A referral means a clinician recommends another service or provider. A referral does not replace required prior authorization. A covered service may still be subject to cost sharing, eligibility, exclusions, and available limits.

Physiotherapy and rehabilitation

Outpatient physiotherapy coverage

Under DEMO-PLAN-001, medically necessary outpatient physiotherapy is covered for up to **20 visits per calendar year** at in-network providers. Physical therapy and physiotherapy refer to the same benefit in this demo plan.

Prior authorization requirement

Prior authorization is required before the first outpatient physiotherapy visit, including the initial evaluation. There is no exception to this requirement for a referral. A physician referral alone is not sufficient authorization.

Cost sharing

An authorized in-network outpatient physiotherapy visit has a **USD 25 member copayment per visit**. No deductible applies to this specific benefit. The 20-visit limit includes the initial evaluation. This document does not establish the billed amount or payment for a particular claim.

Visits beyond the annual limit

Visits beyond 20 are excluded under this demo plan. Prior authorization does not increase the annual visit limit. A member's remaining visit count must be obtained from utilization records, not calculated from this PDF.

Excluded rehabilitation services

Fitness memberships, personal training, sports performance coaching, and rehabilitation equipment purchased for home use are excluded. Occupational therapy and speech therapy are not described by this physiotherapy benefit; their coverage cannot be inferred from it.

Preventive care, office visits, and diagnostics

Annual preventive checkup

One routine preventive health checkup per calendar year is covered at an in-network provider with **USD 0 member cost sharing**. Prior authorization is not required for this checkup. Additional problem-focused services during the same appointment may be billed under a different benefit.

Primary care visits

In-network primary care office visits have a **USD 20 copayment per visit**. Prior authorization is not required. This document does not specify a numerical annual visit limit for primary care; do not invent one.

Specialist office visits

In-network specialist consultations require a referral and have a **USD 40 copayment per visit**. The consultation itself does not require prior authorization. Procedures ordered by a specialist can have separate authorization requirements.

Laboratory testing

Routine diagnostic blood tests ordered by an in-network clinician are covered with a **USD 10 copayment per laboratory encounter**. Prior authorization is not required. Direct-to-consumer wellness panels ordered without a clinician are excluded.

MRI imaging

Medically necessary in-network outpatient MRI imaging requires **prior authorization** and has a **USD 100 copayment per scan**. A specialist consultation referral does not serve as MRI authorization. Other imaging modalities are not specified in this document.

Exclusions, limits, and missing information

Routine dental services

Routine dental examinations, teeth cleaning, fillings, and orthodontics are **excluded** under DEMO-PLAN-001. The preventive health checkup benefit does not include dental checkups. This exclusion does not describe medical treatment of a jaw injury; that situation is not specified here.

Cosmetic services

Elective cosmetic procedures performed solely to change appearance are excluded. Reconstructive surgery following injury is not defined in this document and requires separate benefit verification.

Services not specified

This document does not specify benefits for hearing aids, prescription drugs, maternity care, inpatient admission, occupational therapy, or speech therapy. Absence of a benefit description does **not** establish coverage or exclusion. Request the relevant benefit schedule or contact the demo administrator.

Do not combine unrelated benefit rules

The physiotherapy visit limit applies only to physiotherapy. The USD 0 preventive checkup cost sharing applies only to the annual preventive checkup. Neither rule may be transferred to dental care, MRI imaging, or any other service.

Plan rules versus individual records

This PDF does not contain individual claim statuses, claim IDs, payment dates, member profiles, provider availability, or remaining annual limits. Those questions require a separate authorized record lookup.

Authorization and claims procedures

Requesting prior authorization

For services that require prior authorization, the in-network provider submits the proposed service, diagnosis, requested dates, and supporting clinical documentation before treatment. A routine demo request is reviewed within **five business days after all required documentation is received**. An incomplete request has no review completion date until the missing documents arrive.

Authorization is not a payment guarantee

Authorization confirms review of the proposed service. Payment still depends on active enrollment on the service date, available benefit limits, provider network status, accurate coding, and other applicable plan rules. Approval must not be described as proof that a claim has been paid.

Submitting a claim

For this synthetic plan, submit a claim within **90 calendar days after the service date**. Required documents are an itemized bill, service date, provider identifier, synthetic member identifier, and any required authorization reference. Do not submit real personal or clinical data to this demo application.

Claim status meanings

Pending: review is incomplete or supporting documents are missing.
Approved: the claim has passed adjudication; this does not itself establish payment.
Denied: the claim was not approved; the reason must come from the claim record.
Paid: payment is recorded in the payment system.

Appeal procedure for testing

A synthetic claim appeal may be submitted within **30 calendar days after the denial notice date**, including the claim reference, disputed reason, and supporting documents. This is an invented administrative rule for testing, not a statement of legal rights or real insurance requirements.